INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Carboplatin every 3 weeks with head and neck radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00261
Version 7

> Every 21 days × 3 with radiotherapy

Treatment stages

01 **Carboplatin with radiotherapy**
3 cycles every 3 weeks

02 **Recovery and follow-up**
After concurrent treatment

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for fever, shivering, sudden illness, unusual bleeding, persistent vomiting, much less urine or inability to drink. Report worsening hearing, numbness or tingling before the next dose. Severe breathing difficulty, collapse or severe chest pain is an emergency.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment medicines and modalities

Carboplatin is given with radiotherapy to treat locally advanced head and neck cancer when cisplatin is unsuitable. Your team explains the intended chance of cure or long-term control for your diagnosis.

PLATINUM CHEMOTHERAPY

Carboplatin

intravenous (IV) infusion



Damages deoxyribonucleic acid (the cell's genetic material) (DNA) through cross-links

Carboplatin forms links in DNA that interfere with DNA copying and repair. This can stop cancer cells dividing. Normal blood-forming cells can also be affected, and kidney function determines how the medicine is cleared.

Platinum reaches cells → DNA cross-links form → Copying and repair are disrupted

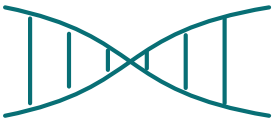
What to watch for

Contact the oncology emergency service immediately for fever or shaking chills, sudden illness, bleeding, new breathlessness, chest pain or severe allergy. Report reduced urine, hearing changes and new numbness promptly. Do not wait for the next visit.

LOCAL TREATMENT

Radiotherapy

Radiotherapy



Damages cancer cells in the planned treatment area

Radiotherapy directs radiation at a carefully planned area. It is delivered on its own appointment calendar. Effects depend on which tissues receive radiation and can build up during the course or continue after it finishes.

Treatment area planned → Repeated local treatment → Recovery and late-effect review

What to watch for

Report difficulty swallowing, weight loss, mouth pain, skin breakdown or problems managing saliva.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your diagnosis and treatment aim

Keep your own diagnosis and treatment aim at the front of the plan.

The setting for this guide

Locally advanced stage III–IV head and neck squamous cell carcinoma when cisplatin is not appropriate.

Choosing between treatments

Your team weighs the cancer, previous treatment, health, symptoms and preferences when discussing options. Other systemic treatment, local treatment where suitable, trials or symptom-focused care may be discussed.

How your treatment works

Cancer treatment can also affect healthy tissue.

Evidence and further information

Use evidence to support a personal treatment discussion.

What studies can tell us

The NCCP regimen includes carboplatin with radiation when cisplatin is inappropriate. Its cited head and neck comparison is a matched-pair analysis; this does not establish randomised equivalence to cisplatin for every patient.

Your individual decision

Group results cannot predict what will happen for you. Ask which benefits matter for your situation and how the team will decide whether treatment remains worthwhile.

Resources

The source list gives the official regimen and public patient information. Ask your team for a printed copy or help accessing a resource if needed.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your overall treatment plan

Treatment stages

01 **Carboplatin with radiotherapy**
3 cycles every 3 weeks

02 **Recovery and follow-up**
After concurrent treatment

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

3 CYCLES EVERY 3 WEEKS

Carboplatin with radiotherapy

1 2 3

Carboplatin is given through a vein over about 30 minutes on Day 1 every 21 days. The head and neck prescription uses AUC 5, a target used with your checked kidney function to calculate the dose. The whole visit includes safety checks, anti-sickness medicines, preparation and observation. Three carboplatin cycles are planned with concurrent radiotherapy. Radiation has a separate appointment calendar; the team coordinates any interruptions or changes.

AFTER CONCURRENT TREATMENT

Recovery and follow-up

Recovery | Individual review

Continue mouth, swallowing, dental, skin and nutrition care after radiation. Blood tests and hearing or nerve review follow any persisting problem; scans and examinations follow your individual oncology and radiation plan.

Three carboplatin cycles are planned with concurrent radiotherapy. Radiation has a separate appointment calendar; the team coordinates any interruptions or changes.

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual cancer-assessment plan

These stages are not equal time intervals. Your team provides dates and reviews results. There is no universal blood tumour-marker panel or scan interval in this guide.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00261 v7: official regimen

Tests in your plan

Full blood count

WHEN
Before starting and before each treatment cycle

Checks blood cells and helps assess infection, anaemia and bleeding risk.

Kidney and liver tests

WHEN
Before starting and before each cycle

Checks organ function; kidney function is also used to calculate carboplatin dose.

Kidney-function measurement

WHEN
Baseline and repeat or reassess when clinically required; estimated dosing needs a current creatinine result.

Provides the renal value needed for the carboplatin dose calculation; measured function is preferred when feasible.

Scans and head and neck examination

If part of your plan

WHEN
As prescribed in the individual disease-assessment plan

Assesses the cancer and the response to treatment separately from safety bloods.

THE WHOLE TREATMENT JOURNEY

Carboplatin every 3 weeks with head and neck radiotherapy: treatment and checks

Carboplatin is given through a vein over about 30 minutes on Day 1 every 21 days. The head and neck prescription uses AUC 5, a target used with your checked kidney function to calculate the dose. The whole visit includes safety checks, anti-sickness medicines, preparation and observation. Three carboplatin cycles are planned with concurrent radiotherapy. Radiation has a separate appointment calendar; the team coordinates any interruptions or changes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response review	After treatment
Bloods and organ safety NCCP	Full blood count, kidney and liver tests; review kidney-function measurement, earlier platinum reactions, nerve and hearing symptoms and all medicines.	Full blood count, kidney and liver tests before every cycle. The team verifies the current kidney result and dose calculation, and reviews infection, bleeding, hearing and nerve symptoms.	New symptoms may require extra tests before the next planned review.	Safety bloods do not by themselves show whether treatment controls the cancer.	Continue mouth, swallowing, dental, skin and nutrition care after radiation. Blood tests and hearing or nerve review follow any persisting problem; scans and examinations follow your individual oncology and radiation plan.
Concurrent radiotherapy NCCP	Dental, nutrition and swallowing preparation; obtain a separate radiation calendar.	Attend radiation on the prescribed calendar; it is not given only on carboplatin days.	Tell the team about missed sessions, skin reactions and worsening mouth or swallowing symptoms.	The radiation and oncology teams coordinate changes.	Recovery and dental or swallowing effects may continue after the final radiation session.
Head and neck function NCCP	Record weight, intake, swallowing, speech and existing mouth or dental problems.	Review pain, intake, weight and support needs.	Report choking, inability to drink, worsening swallowing or new neck symptoms.	Agree examinations and scans with the oncology team.	Continue rehabilitation and symptom support as needed.
Medicines and practical plan NCCP	Confirm the exact prescription, treatment aim and actual urgent contacts.	Bring your medicine list, current support plan and treatment record.	Record symptoms and medicines taken; ask before adding a new product.	Confirm any pause, dose change or stop in writing.	Record the final dose and the teams responsible for follow-up.

What each check is for

1 **Carboplatin with radiotherapy**

3 cycles every 3 weeks

Carboplatin is given with radiotherapy to treat locally advanced head and neck cancer when cisplatin is unsuitable. Your team explains the intended chance of cure or long-term control for your diagnosis.

Ask / act: Confirm the current phase, prescription, tests and support plan.

2 **Recovery and follow-up**

After concurrent treatment

Carboplatin is given with radiotherapy to treat locally advanced head and neck cancer when cisplatin is unsuitable. Your team explains the intended chance of cure or long-term control for your diagnosis.

Ask / act: Confirm the current phase, prescription, tests and support plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan



STEP 3

Tests and review

Full blood count, kidney and liver tests before every cycle. The team verifies the current kidney result and dose calculation, and reviews infection, bleeding, hearing and nerve symptoms.



STEP 4

Home record

Carry your treatment record, actual urgent contacts and medicine list. Tell the team about line redness, swelling, leakage, pain or fever.



STEP 5

Follow-up

Continue mouth, swallowing, dental, skin and nutrition care after radiation. Blood tests and hearing or nerve review follow any persisting problem; scans and examinations follow your individual oncology and radiation plan.

Your treatment schedule

Keep one calendar for treatment, tests and other appointments.

A pause or change

Blood results, kidney function or side effects may delay or change a dose. The chemotherapy and radiation teams coordinate the plan; do not use an old appointment list as authority to restart.

Medicines, precautions and visit preparation

Review medicines and practical needs before treatment.



Medicines and fertility

Bring prescribed, over-the-counter and herbal medicines and supplements for review. Discuss fertility preservation and breastfeeding before treatment. Carboplatin can impair fertility. Do not breastfeed during treatment. The linked Irish product information advises contraception for women during treatment and at least six months afterwards; men should avoid fathering a child during treatment and at least three months afterwards. Confirm the actual product and personal advice.

Access and observation

Your team chooses suitable venous access for an infusion. A port, if used, has its own insertion, care and removal plan; it is not removed after each visit. No take-home chemotherapy pump is part of this regimen.

Work, transport and communication

Discuss transport, work, caring responsibilities, interpreter needs and a support person. Ask how results and changes will be communicated and who books your next tests.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Carboplatin treatment	 **Day 1** Carboplatin is given through a vein over about 30 minutes on Day 1 every 21 days. The head and neck prescription uses AUC 5, a target used with your checked kidney function to calculate the dose. The whole visit includes safety checks, anti-sickness medicines, preparation and observation.
Radiotherapy and symptom support	 **Days 1–21** Radiation follows its own calendar; the band shows overlap, not daily treatment. Mouth and swallowing symptoms can build across the course.
Prepare next review	 **Day 21** Arrange required tests before the next dose; timing and results must be confirmed by your team.

Treatment sequence: Three carboplatin cycles are planned with concurrent radiotherapy. Radiation has a separate appointment calendar; the team coordinates any interruptions or changes.

After treatment: Continue mouth, swallowing, dental, skin and nutrition care after radiation. Blood tests and hearing or nerve review follow any persisting problem; scans and examinations follow your individual oncology and radiation plan.

Why these bands stay broad: This is an illustrative cycle, not a prediction of safe days or a substitute for the appointment calendar. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00261 v7: official regimen ↗

There are no guaranteed “safe days”. This information supports discussion; your oncology team confirms the personal prescription and urgent-care plan.

Between-treatment notes

Day 1: Treatment visit

Carboplatin is given through a vein over about 30 minutes on Day 1 every 21 days. The head and neck prescription uses AUC 5, a target used with your checked kidney function to calculate the dose. The whole visit includes safety checks, anti-sickness medicines, preparation and observation. Report symptoms and previous reactions before treatment.

Day 2–21: Between visits

Radiation has its own calendar. Mouth, skin and swallowing effects can build across weeks and continue afterwards. Follow the support plan and contact oncology early if symptoms develop.

Support around the medicines

1 **Anti-sickness medicines**

This carboplatin regimen has high sickness risk. Take the prescribed prevention and rescue medicines on their written schedule. Call if nausea or vomiting prevents drinking.

2 **Eating and weight**

Tell the team early if swallowing, appetite, pain or taste changes reduce intake. A dietitian can agree practical food, drink or feeding support with you. Do not wait for substantial weight loss.

3 **Mouth and dental care**

Agree a mouth-care plan and tell your dentist about previous or current cancer treatment. Arrange oncology advice before dental procedures, particularly after head and neck radiation.

4 **Swallowing and communication**

Ask for speech and language therapy if swallowing or speech is difficult. Use the food textures, exercises and communication support advised for you; a general guide cannot establish safe swallowing.

5 **Medicines and fertility**

Tests and review

Blood safety tests, tumour tests and scans answer different questions.

Before the first dose

Full blood count, kidney and liver tests; review kidney-function measurement, earlier platinum reactions, nerve and hearing symptoms and all medicines.

Response assessment

Your team agrees examinations and scans to assess cancer response. No universal scan interval or blood tumour-marker panel is assumed here.

Recognising side effects

Use the anatomical map with these explanations; location does not tell you how urgent a symptom is.

Hearing / tinnitus

Report hearing change or ringing in the ears; previous platinum treatment increases risk.

Mouth / throat / saliva

Mouth pain, thick saliva, dry mouth or swallowing difficulty can limit drinking; seek help early.

Blood cells / infection

Fever, shivering, sudden illness, unusual bruising or bleeding needs prompt oncology advice.

Kidneys / fluids

Kidney tests guide the carboplatin dose. Much less urine, persistent vomiting or inability to drink needs urgent advice.

Nausea / appetite

Take the prescribed prevention and rescue medicines. Call if vomiting persists or fluids will not stay down.

Nerves / hands / feet

Report numbness, tingling, balance problems or difficulty with fine tasks; symptoms can persist.

Skin / treated area

Follow the radiation team’s skin plan. Report painful redness, peeling or a wound that is not healing.

Teeth / jaw / swallowing

Dental review, nutrition and swallowing support continue through recovery. Report jaw stiffness, weight loss or choking.

When and how to get help

Use the oncology service’s written urgent-contact plan.

During an infusion or injection

Tell staff immediately about itching, a rash, dizziness, chest tightness, breathing difficulty, pain or swelling around the treatment site.

Prepare the handover

Tell the clinician the medicine name, date of the last dose, new symptoms and any prescribed steroids or hormone replacement. Carry the wallet card; do not delay urgent care while looking for a document.

Eating, mouth care and everyday support

Ask for help before a practical problem prevents treatment.

Anti-sickness medicines

Eating and weight

Swallowing and communication

Ask for individual speech and language therapy advice for swallowing or communication difficulties. Report choking, coughing with drinks or inability to swallow safely.

Energy and activity

Plan manageable activity and rest around symptoms. Tell the team about severe or unexpected fatigue, falls or difficulties managing at home.

Your record and contact card

Keep the information that another clinician would need.

Cycle or review record

Record the cycle received, date, route, dose if supplied, important symptoms, any hold and the next agreed review. Stamp boxes record treatment received; they do not authorise the next dose.

Contacts and documents

Have the team replace example telephone numbers with your actual daytime and out-of-hours contacts. Carry the front/back wallet card and immune alert card when applicable. The printed handbook contains the key information without needing to scan a QR code.

Questions for review

Bring your main question, changes in swallowing or weight, symptoms, medicine changes and any concerns about the treatment aim. Ask for the plan in words you can explain back.

Questions worth asking

What happens if treatment is held?

The team records the reason, required tests and recovery and who can authorise restarting. Do not count a held dose as received.

Try explaining the plan back: Explain your treatment schedule in your own words.,Name the symptoms that mean you should call.,Point to the actual urgent contact number and the next required tests.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Carboplatin with radiotherapy

Active medicines: Carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on Day 1; 80 mg orally on Days 2 and 3	Once daily on each listed day Check interactions; follow the prescribed plan.
Ondansetron	16 mg orally on Day 1	Before treatment NCIS permits individual splitting to 8 mg twice daily or reduction to 8 mg once daily. Review QT risk, electrolytes and constipation.
Dexamethasone	12 mg orally on Day 1; 8 mg orally on Days 2, 3 and 4	Once daily on each listed day Confirm timing and diabetes/sleep advice. This is the dedicated carboplatin row.
Metoclopramide · duration requires prescription	10 mg orally three times daily on subsequent days in the NCIS row	The source row does not specify an end day or PRN qualifier Prescriber/pharmacist must specify duration and suitability, including renal function and neurological contraindications. Do not continue indefinitely.

Additional phase-specific support

- This carboplatin regimen has high sickness risk. Take the prescribed prevention and rescue medicines on their written schedule. Call if nausea or vomiting prevents drinking.

- Tell the team early if swallowing, appetite, pain or taste changes reduce intake. A dietitian can agree practical food, drink or feeding support with you. Do not wait for substantial weight loss.
- Agree a mouth-care plan and tell your dentist about previous or current cancer treatment. Arrange oncology advice before dental procedures, particularly after head and neck radiation.
- Ask for speech and language therapy if swallowing or speech is difficult. Use the food textures, exercises and communication support advised for you; a general guide cannot establish safe swallowing.
- Bring prescribed, over-the-counter and herbal medicines and supplements for review. Discuss fertility preservation and breastfeeding before treatment. Carboplatin can impair fertility. Do not breastfeed during treatment. The linked Irish product information advises contraception for women during treatment and at least six months afterwards; men should avoid fathering a child during treatment and at least three months afterwards. Confirm the actual product and personal advice.
- Continue mouth, swallowing, dental, skin and nutrition care after radiation. Blood tests and hearing or nerve review follow any persisting problem; scans and examinations follow your individual oncology and radiation plan.
- NCIS Table 1 allows clinician-selected ondansetron splitting to 8 mg twice daily or reduction to 8 mg once daily according to the individual patient.
- Review ondansetron QT risk and other QT-prolonging medicines/electrolytes.
- Check dexamethasone interactions, diabetes, sleep and infection effects; aprepitant/steroid interaction is already reflected in the specified row.
- Review metoclopramide renal function, neurological contraindications, duration and extrapyramidal risk; do not automatically stack with olanzapine.
- AUC reduction below 4 requires an individual prophylaxis review; history of emesis and patient factors may justify continuing stronger protection.
- No universal granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or acid-suppression prescription is stated by the carboplatin protocol. G-CSF requires individual febrile-neutropenia risk review.
- Routine hypersensitivity premedication is usually unnecessary, but antiemetic prophylaxis is still required. Prior allergy/desensitisation is a separate specialist plan.
- Use the dedicated carboplatin AUC ≥4 row. It does not include routine olanzapine.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hearing / tinnitus

Carboplatin

Report hearing change or ringing in the ears; previous platinum treatment increases risk.

3 Blood cells / infection

Carboplatin

Fever, shivering, sudden illness, unusual bruising or bleeding needs prompt oncology advice.

5 Nausea / appetite

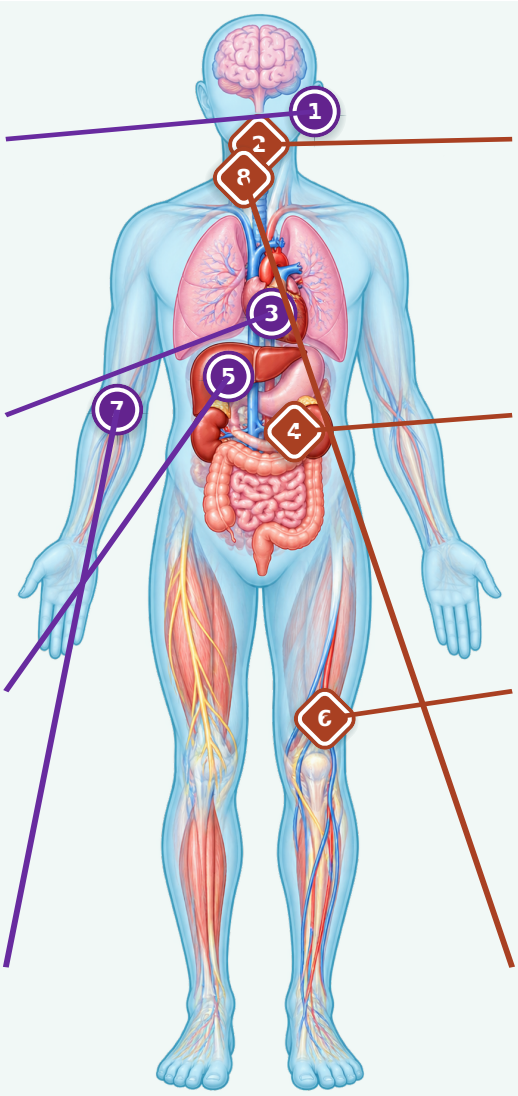
Carboplatin

Take the prescribed prevention and rescue medicines. Call if vomiting persists or fluids will not stay down.

7 Skin / treated area

Radiotherapy

Follow the radiation team's skin plan. Report painful redness, peeling or a wound that is not healing.



2 Mouth / throat / saliva

Head and neck radiotherapy

Mouth pain, thick saliva, dry mouth or swallowing difficulty can limit drinking; seek help early.

4 Kidneys / fluids

Carboplatin

Kidney tests guide the carboplatin dose. Much less urine, persistent vomiting or inability to drink needs urgent advice.

6 Nerves / hands / feet

Carboplatin

Report numbness, tingling, balance problems or difficulty with fine tasks; symptoms can persist.

8 Teeth / jaw / swallowing

Head and neck radiotherapy

Dental review, nutrition and swallowing support continue through recovery. Report jaw stiffness, weight loss or choking.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology promptly for fever, shivering, sudden illness, unusual bleeding, persistent vomiting, much less urine or inability to drink. Report worsening hearing, numbness or tingling before the next dose. Severe breathing difficulty, collapse or severe chest pain is an emergency.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00261 v7 · pNCCP 00261 v7 p4; NCIS antiemetics v8 Table 1 pp4–5**
   https://healthservice.hse.ie/documents/6366/261_CARBOplatinAUC4-6_21days.pdf
2. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **Current Irish product information · safety supplement**
   https://assets.hpra.ie/products/Human/28088/Licence_PA2059-032-001_02072024153224.pdf
6. **NCCP NCIS antiemetic medicines v8 · dedicated carboplatin AUC ≥4 row, pages 4–5**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf